جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه صالح احمد مصطفي
المحافظة : الجيزه **الحي / المدينة :** الاخصاص **العنوان :** ي
رقم البطاقة : 25401312100842 **رقم التليفون :** 01098954789 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 31-01-1954 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه صالح احمد مصطفى
National ID: 25401312100842

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

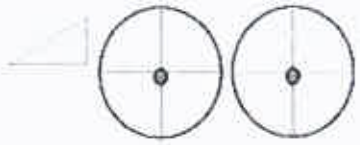
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سامية صلاح احمد مصطفى Sex: Female
3- Medical number: 83167572
Old Med : 3861-2023

2-ID number: 25401312100842 Patient Age : 70 Y



Family history	Family History	Phone	Phone 01098954789	BMI	BMI 9	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP A	Date of MDT	Date Of MDT 16/12/2023
Co morbidity	Co Morbidity htn	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup : ct cap:tiny P.N

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT: gli lt focal	7/8 ER COMMENT:	7/8 PR COMMENT:	Her 2 Neu COMMENT: -ve	SISH COMMENT :	15

T	N	M
P1	1	0

Sonomammography

PATIENT HISTORY

LT MRM: MIXED DUCAL & LOBULAR CARCINOMA PT1N1/11, WITH DCIS SEEN IN LESS THAN 1MM FROI
DEEP RESECTION MARGIN
PELVIC MASS EXCISION: AFTER IHC IN HARMEL CELLULAR ANGIOFIBROMA
(BAHEYA REPORTED DFSON BX)
NO COMMENTY ON MARGIN ON SURGICAL EPECIMEN OF PELVIC MASS IN ELMOKHTABAR

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR FOR SURGICAL CONS. FOR POSITIVE MARGINS (DCIS) IF NO FURTHER SURGER FOR 6 CYCLES TC (TAXOTERE/ENDOXAN) ,PORT, AI FOR 5 YEARS
COMMENT ON SURGICAL MARGINS FOR PELVIC MASS FROM ELMOKHTABAR

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ ماجد الفاروق	RADIOLOGY	RADIOLOGY د/ شعن
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / ساميه صالح احمد مصطفى عبده	Request No.	: 13004112
Patient No.	: 25401312100842	Request Date	: 18/11/2023
Path No.	: SR-1420/23	Verification Date	: 02/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 69 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
Left breast mass

Nature of specimen
Revision of slides
Gross examination
15 blocks (3149023) &15 slides were received .

Microscopic examination
Breast mass
* Histological type: Invasive duct carcinoma with fixation artifacts
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
* DCIS: Focal 20%
* Angioinvasion: Could not be detected within the limits of biopsy received
*Lymph nodes: positive in one lymph nodes (1/11)
IPT :
ER:Intermediate nuclear staining in 70% of tumor cells(7/8)
PR:Intermediate nuclear staining in 70% of tumor cells (7/8)
Her2: No memrane staining in tumor cells , score(0)
KI67:Positive in 15% of tumor cells
LOIN MASS:
microscopic examination revealed lesion with collagenous stroma with nicreased vasculature with few spindle cells
CD68:few positive cells
SOX10:Negative
KI67 posistive cells < 1% of tumor cells

Diagnosis

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ساميه صالح احمد مصطفى عبده	Request No.	: 13004112
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Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Left breast mass ; slide revision :
Invasive duct carcinoma, Grade II with focal DCIS 20%(with fixation artifacts)
IPT :
ER: Positive (score 7/8)
PR:Positive (score 7/8)
Her2: Negative score(0)
Ki67:Positive in 15% og tumor cells
Loin mass: slide revision :
cellular angiofibroma, no malignancy

Signature
Dr. Iman Gouda
Dr.Heba Hassan
Dr .Menna Nabil

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ساميه صالح احمد مصطفى عبده	Request No.	: 13004458
Patient No.	: 25401312100842	Request Date	: 12/10/2023
Path No.	: SR-1239/23	Verification Date	: 30/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 69 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Slide Revision

Gross Examination:
- 1 paraffin block labeled by (203303) and 3 slides were received.
- 1 paraffin block labeled by (7073) and 5 slides were received.

Microscopic Examination:
- Slide (2):
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Focal.
* Angioinvasion: No.
* Perineural invasion: occasional.
* IPT:
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8 /8).
- PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8 /8).
- Her2: No membrane reaction in the tumor cells (score 0).

Slide (1):
Spindle cell lesion.
IPT:
Ki67: 3%.
SMA: Negative (Positive internal control).
B catenin: Focal Positive cytoplasmic only , no nuclear staining.
CD34: Positive.
S100: Negative.

Verified by :
DR.Menna Nabil

CAP ACCREDITED
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ساميه صالح احمد مصطفى عبده	Request No.	: 13004458
Patient No.	: 25401312100842	Request Date	: 12/10/2023
Path No.	: SR-1239/23	Verification Date	: 30/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 69 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:
Left breast cancer, Slide revision:
Slide (2):
Invasive duct carcinoma GII with focal DCIS.
IPT:
- ER: Positive (score 8 /8).
- PR: Positive (score 8 /8).
- Her2: Negative (score 0).

Slide (1):
Compatible with myofibroblastoma.

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :
DR.Menna Nabil

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

AL SHROOUK SCAN
الشروق للأشعة

15184

الاسم :.	سامية صالح احمد	تاريخ الفحص :.	26/7/2023
السن :.	65	كود المريض :.	19230
جهة التحويل :.		رقم العملية :.	31570
الطبيب المعالج أ.د.:.	مروه اسامه الديب	*Thank you for referring this case,*	

Thanks for reference

Bilateral digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

Right breast:

- Scattered mixed densities of glandular condensations are noted (ACR-B).
- Vascular atheromatous calcifications are noted.
- No speculated masses or pathological microcalcifications.
- Normal skin thickness & contour.
- Echogenic mammary parenchyma.
- Multiple intramammary LNs are noted ,measuring 3-6 mm in diameter.
- Right LOQ well circumscribed oval shaped hypoechoic lesion is seen at 8 O'clock ,para-areolar in location ,measuring 8x4 mm in diameter ,being avascular upon Color Doppler application.
- No architecture distortion.
- Large sized right axillary lymph nodes are noted ,measuring 19x9 mm in diameter with normal cortical thickness and preserved fatty hilum.
- Right axillary accessory breast is noted.

Left breast:

- Scattered mixed densities of glandular condensations are noted (ACR-B).
- Left breast LOQ spiculated outline dense mass lesion is noted ,seen by U/S as irregular shaped hypoechoic solid mass lesion at 7 O'clock ,about 3 cm from the areola ,measuring 17x14 mm in diameter with infiltration and fixation to the overlying skin and marked surrounding architectural distortion ,being avascular upon Color Doppler application.
- Focal thickening of the overlying skin is noted.

Mammogram
[DOCTYPE:MAMMOGRAM]

الاسم :	سامية صالح احمد	تاريخ الفحص :	26/7/2023
السن :	65	كود المريض :	19230
جهة التحويل :		رقم العملية :	31570
الطبيب المعالج أ.د.:	مروه اسامه الديب	*Thank you for referring this case,*	

- Two other similar lesions are seen at :
  - 5 O'clock ,about 5 cm from the areola ,measuring 19x13 mm in diameter.
  - 8 O'clock ,about 5 cm from the areola ,measuring 18x16 mm in diameter.

- Left LIQ segmental area of architectural distortion with multiple clusters of pathological microcalcifications are noted ,seen by U/S as regional area of altered echogenicity at 6 O'clock ,about 4 cm from the areola ,measuring 18x11 mm in diameter with microcysts within ,being avascular upon Color Doppler application.
- Another similar lesion is seen at the retroareolar region ,4-5 O'clock ,measuring 13x7 mm in diameter.
- Multiple pathologically enlarged intramammary LNs are noted measuring 5-7 mm in diameter with the one at 3 O'clock ,para-areolar in location and 4 O'clock ,about 2 cm from the areola ,showing surrounding dysplastic reaction.
- Vascular atheromatous calcifications are noted.
- Large sized left axillary lymph nodes reaching 14x8 mm in diameter with normal cortical thickness and preserved fatty hilum.
- No pathological infra or supraclavicular LNs.
- Left axillary accessory breast is noted.

Mammogram
[DOCTYPE:MAMMOGRAM]

PATIENT NAME: **SAMIA SALEH AHMED MOSTAFA** GENDER: **Female** AGE: **69 y**	PATIENT ID #: **10123229403** ORDER #: **2472251,2472252** DATE OF SERVICE: **16-Sep-2023 01:35 PM**

Report: - all studies

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

1-CHEST

- Left breast LIQ ill defined heterogenous enhancing soft tissue nodules, the largest is seen infiltrating the overlying thickened skin measuring about 16x18 mm. few tiny outer quadrants enhancing nodules measuring about 6 mm.
- Solitary prominent left axillary lymph node is seen measuring about 10 mm.
- Multiple bilateral tiny pulmonary nodules, the largest is seen at left lower lung lobe measuring about 5 mm (partially calcified nodule).
- No pathologically enlarged hilar and mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Enlarged thyroid gland showing heterogenous bilobar nodules and coarse calcifications.

2- ABDOMEN AND PELVIS:

- The liver is enlarged in size with homogenous CT texture. No definite hepatic focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- Relatively small size left kidney.
- Bilateral renal lobulated outlines/ irregular cortical scarring.
- Bilateral renal cortical cysts.
- Bulky uterus for age.
- Urinary bladder stone is seen measuring about 9 mm.
- Normal appearance of the pancreas, spleen, adrenals, aorta, I.V.C and

[[DOCTYPE:CT_SCAN]]

the other pelvic organs.

- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- Left femoral DHS placement.
- **The revised bone window revealed:**

1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
2. Right posterior iliac region well defined subcutaneus enhancing soft tissue lesion is seen measuring about 35x 30 mm.
3. Two other small subcutaneous nodules seen at right shoulder as well as overlying left trapezius muscle posteriorly.

Opinion:

A reported case of left breast cancer showing:

- ***Multiple bilateral tiny pulmonary nodules, for follow up.***
- ***Right posterior iliac region well defined subcutaneous enhancing soft tissue mass lesion; likely metastatic.***

Reported By,

Dr. Ayman Elaraby, MD
Dr. Ghada Hamed Khalifa,MSc **17-Sep-2023 06:32 PM**

CT Scan
[[DOCTYPE:CT_SCAN]]